

Patient Safety Toolkit

Core Patient Safety Goals

- Correctly identify every patient using two identifiers before any procedure, medication, or transfusion.
- Improve communication among caregivers, especially during shift handovers, using the SBAR format (Situation, Background, Assessment, Recommendation).
- Reduce the risk of healthcare-associated infections through hand hygiene compliance at all five WHO moments.
- Reduce the risk of harm from falls by screening every inpatient with a validated fall-risk tool on admission.

Medication Safety

- High-alert medications (insulin, heparin, opioids, concentrated electrolytes) require an independent double-check by a second nurse before administration.
- All verbal orders must be read back and confirmed before being carried out.
- Look-alike/sound-alike (LASA) drugs are stored separately and labeled with tall-man lettering.

Incident Reporting

- Any near-miss, adverse event, or sentinel event must be reported through the hospital's electronic incident reporting system within 24 hours.
- Reports are reviewed without individual blame, focusing on system-level causes, in line with a 'just culture' approach.

Surgical Safety

- The WHO Surgical Safety Checklist is completed at three points: before anesthesia (sign-in), before incision (time-out), and before the patient leaves the operating room (sign-out).